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Steven Posnack, MS, MHS
Principal Deputy Assistant Secretary for Technology Policy
Principal Deputy National Coordinator for Health Information Technology
The Office of the National Coordinator for Health IT
U.S. Department of Health and Human Services
330 C Street, SW, Floor 7
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Principal Deputy Posnack:

On behalf of the physician and medical student members of the American Medical Association (AMA), I appreciate the opportunity to provide feedback to the U.S. Department of Health and Human Services (HHS) on its Request for Information regarding accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. The AMA commends the Administration for its consideration and commitment to leading the charge in exploring how AI technologies present unique opportunities in health care to enhance care delivery, strengthen patient-provider relationships, reduce administrative burdens for clinicians, and empower patients to navigate the health care system better and get the care they need. Currently, however, the unresolved questions about liability, regulatory authority, patient safety, and state oversight are converging, making AI governance one of the most consequential issues facing health law and medical practice. A growing liability vacuum has emerged as AI developers increasingly disclaim responsibility for errors or harm, even as clinicians are expected to rely on these tools in clinical and administrative decision-making. Regulatory ambiguity, unresolved coverage and reimbursement questions, data privacy risks, and limited public trust are inhibiting rapid adoption, highlighting the need to modernize oversight frameworks that were not designed for the scale, speed, or complexity of AI-enabled care.

The AMA asserts that foundational concepts, measures, and principles are crucial for the successful integration of AI into the health care system and the practice of medicine. Physicians must play a central role in the ethical development, deployment, and utilization of AI technologies in health care, as their clinical expertise is indispensable in ensuring these tools are safe, effective, and trustworthy. Establishing strong digital health foundations, including telehealth infrastructure, robust privacy and security protections, and seamless interoperability, is critical to enabling this transformation. When implemented responsibly, AI-powered technologies hold significant potential to enhance patient-centered care, improve clinical outcomes, and reduce costs.

The AMA has included key areas of consideration:

Regulation of Health Care AI

The AMA appreciates HHS' interest in continuing to evaluate regulatory approaches to health care AI. As you know, appropriate oversight structures for health care AI will play a significant role in ensuring AI with clinical applications is safe and efficacious, promotes innovation, and, most importantly, builds trust between developers, physicians, and patients. In a survey of over 1,000 physicians across specialties and practice settings, appropriate regulatory oversight was identified as one of the top needs of physicians when considering whether to integrate AI into their practice.

The AMA has long supported movement towards a harmonized national governance approach for health care AI. This type of approach has, to date, been lacking and would provide developers, deployers, physicians and patients with clarity and consistency in a way that can help enhance understanding and build much needed trust in AI-enabled health care technologies. At a basic level, terminology has not been harmonized across the federal government, with agencies such as the U.S. Food and Drug Administration (FDA), the Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology , the Office for Civil Rights (OCR), and the Centers for Medicare & Medicaid Services (CMS) all utilizing very different terminology to describe what frequently are many of the same technologies and functions. Stakeholders including industry participants, physicians, and patients have long advocated for consistent regulatory terminology across HHS to help alleviate confusion and provide consistency across agencies. We would recommend HHS consider this need for harmonization as they move forward with any updated regulatory approach towards health care AI.

At a higher level, the AMA has consistently advocated that regulation of health care AI, particularly AI with clinical applications, should follow a risk-based approach. There are likely a number of AI-enabled technologies that pose a low risk to patient health. These likely do not need stringent regulatory scrutiny before they come to market. However, AI that poses higher levels of risk to patients needs appropriate regulatory scrutiny to ensure it is safe and performs as promised with appropriate guardrails. Every AI-enabled technology that impacts medical decision-making should be required to have a rigorous post-market surveillance plan to help ensure continued performance. Developers and deployers of these technologies should be able to identify algorithmic drift and ensure their algorithms are not introducing bias at any point in time. FDA-regulated device software functions, even those falling under enforcement discretion, should have mandated labeling requirements to help provide the end user with essential transparency about the product in question. **The AMA feels strongly that a clear, consistent, risk-based approach to regulation in lieu of a significant deregulatory approach is required to enhance trust in AI and successfully move towards increasing clinical integration.**

Transparency

A critical element of successful risk-mitigation that can help to enhance trust in AI is transparency. The AMA recommends that certain AI tools be subject to transparency mandates that clearly outline the intended use of the tool, how to use the tool, what risks and limitations may exist, and other elements. For AI with clinical applications, AMA policy recommends that specific elements be disclosed to physicians so they can make informed decisions about purchasing and using AI-enabled tools in practice. For regulated software device functions, this could come in the form of a labeling requirement. We were pleased to see the FDA's January 2025 draft guidance titled "Artificial Intelligence-Enabled Device Software Functions: Lifecycle Management and Marketing Submission Recommendations" includes recommendations for updated labeling appropriate for software device functions, including AI. We urge the Administration to finalize guidance regarding labeling requirements as it continues to evaluate its approach to AI regulation. Furthermore, we urge the FDA to recommend that this apply to software device functions for which they plan to exercise enforcement discretion. For tools like clinical decision support software, where physicians will need to be able to review the basis for the software's recommendations, a consistent approach to transparency is essential.

Transparency plays an essential role in helping physicians successfully navigate AI use in their practice. Transparency is necessary to help physicians understand the product they are using and select the right tool for use with the right patient at the right time. While we expect that some physicians will develop a significant amount of expertise in AI over time, expecting all physicians to have a deep understanding of how AI-enabled technologies function is likely unreasonable. This is increasingly important for physicians in small practices or underserved areas who will not have the evaluation resources that large health systems and hospitals do. Because of this, developers and deployers should provide information about their products in a way that is understandable to physicians of all levels of experience with AI. We also suggest a standardized format for disclosures across industry participants. A consistent approach to transparency makes it easier for all physicians to understand and compare technologies, helping them make better choices about which tools they engage with in their practice.

While the FDA has a large role to play in ensuring transparency for software device functions, other technologies in the health care space should provide some level of transparency as well. While FDA authority does not extend to many administrative AI tools, direct-to-consumer chatbots and other wellness applications, for example, the AMA believes it is important that certain information is disclosed to users of those tools. These lower-risk tools do not likely need to provide the significant level of transparency that would be required of a medical device. However, it is important that end users, including consumers, understand the tool's intended use, its limitations, and that the user is not interacting with a human. The Federal Trade Commission (FTC) would be the most appropriate regulatory body to address transparency issues related to consumer protection. While we understand that FTC's work is not under the purview of HHS directly, we encourage HHS to work with FTC to consider how they might provide guidance to industry, physicians, and patients regarding transparency requirements furthering consumer protections.

Regulation of Consumer-Facing Autonomous AI

New direct-to-consumer health care technologies are also being introduced to consumers at a rapid pace. These technologies hold significant promise to help increase patient understanding of their own health and wellness, improve health literacy, and potentially expand access to care by making certain services more accessible and affordable. However, many of these technologies will seek to operate in an autonomous manner, including many AI chatbots and AI agents. These technologies are incredibly promising but also carry risks that we should work to mitigate.

Autonomous AI will need appropriate guardrails, oversight, and transparency. Consumers should have a very clear understanding that they are not interacting with a human through regulatory mandates requiring clear disclosure that a patient is interacting with AI. Transparency mandates should include prohibitions on any claims or insinuations that a chatbot or agent is a licensed clinician or can otherwise provide the same services as licensed clinicians. Regulators, industry, physicians, and patients should work together to develop clear and enforceable guardrails that mitigate risks and establish the appropriate scope of activities in which autonomous AI can safely be a participant. The AMA appreciates and supports the FTC's investigation into AI chatbots raising mental health concerns and encourages HHS to work with Congress and the FTC to ensure appropriate guardrails are in place to mitigate harm to consumers that may result from engaging with these technologies.

AI "Deep Fakes" and Impersonation of Physicians

Advances in generative AI now allow realistic impersonation of physicians through synthetic images, audio, and video ("deepfakes"). When used maliciously or without consent, these tools can spread medical misinformation, damage physicians' reputations and careers, and undermine patient trust in evidence-based care. Existing privacy, employment, and intellectual property laws do not adequately address these risks in clinical contexts.

This is about patient safety, not technology restrictions. Medical deepfakes can convince patients to follow harmful advice, delay care, or abandon evidence-based treatment. Consumers cannot reasonably distinguish real clinicians from AI-generated impersonations without clear protections and labeling. Patients rely on trust signals they cannot independently verify, such as a physician's voice, image, and credentials, which function as trusted cues for consumers. Deepfakes exploit those cues, creating a uniquely high-risk form of consumer deception in health care.

When considering this issue, we urge the Administration to work with Congress in developing protections that focus on key elements that protect the patient/consumer:

- Preventing impersonation reduces health misinformation at the source.
- Shared responsibility protects consumers more effectively.
- Deepfake misuse disproportionately harms vulnerable patients.
- Transparency supports innovation and trust.
- Patients benefit from default protections.
- **Protecting physicians ultimately protects patients.**

Physician Liability for Use of AI

The AMA is pleased to see HHS' acknowledgment that the use of AI in clinical care raises novel and complex legal questions for patients, physicians, hospitals, health systems, and malpractice insurers to contemplate. While HHS has inquired about legal issues surrounding non-device AI, the issue of liability crosses both device and non-device AI-enabled technologies. The issue of liability is particularly important to physicians as they begin to engage with AI in their practices in a more meaningful way. In the AMA's AI Physician Sentiment Survey, respondents identified concerns around liability for use of AI as one of their top concerns when considering whether to implement these technologies into their practices. The question of who is responsible for medical errors that involve the use of AI is currently without precedent in case law, and no prevailing legal frameworks exist to address the issue, making it a difficult situation for physicians to navigate and one that could have significant implications for the successful clinical integration of AI-enabled technologies. The use of AI that impacts patient-physician medical decision-making does not fall neatly within current legal constructs, such as approaches to product liability. Should physicians bear a disproportionate amount of risk of liability for medical errors where poorly performing AI played a role in medical decision-making, it could act as a disincentive to use the tools in clinical care. Increases in liability risks for physicians may also impact malpractice insurance, potentially raising premiums, increasing deductibles, or resulting in policy riders that specifically include or exclude certain uses of AI.

As we move toward more personalized AI, it may ultimately become a meaningful participant in medical decision-making. AI that can capably evaluate personalized health information, including information extracted from individual medical records, diagnostic results, images, personal health data from wearables, etc., presents a situation in which human physicians and algorithms are together engaged in a new type of shared decision-making. While these capabilities will represent a significant step towards realizing AI's potential to improve clinical care, they also present a novel risk to physicians and patients—what if the AI is wrong? If an AI tool is marketed as capable of performing accurately in a way that a physician cannot, it can play a valuable role. However, if the AI returns an incorrect output, that error can have serious consequences for patients if it contributes to misdiagnosis or improper treatment. In this scenario, who should bear responsibility and any resulting harm?

In scenarios where AI will contribute to diagnosis and care decisions, AMA policy states that the developer of that technology should be accountable for its role in care decisions that are impacted by its performance. This is especially true when the algorithms underlying the technologies are "black box," meaning that physicians and patients cannot fully understand the logic or rationale leading to the outputs. If a reasonable physician did not know, or did not have reason to know, that an AI error had occurred when it should have been reasonable to rely on the outputs of a particular technology, the developer of the technology should be accountable to patients for the resulting harm. Should developers of AI technologies have no accountability for the performance of their tool, there will be very little incentive for physicians to meaningfully engage with them, knowing they bear all the risk for its performance. Claims that physicians should never rely on the outputs of the technology would seem to obviate the need for utilizing the tool in the first place or would result in utilizing them as a confirmatory tool only.

Physicians will still bear the burden of ensuring they are appropriately educated in the selection and use of AI technologies and are validating the outputs to the extent possible. Additionally, should the physician be negligent in tool selection and use, such as selecting the wrong tool, using it on the wrong patient, or using it outside its intended use, they should be accountable for those actions. However, it may not be possible for physicians to validate the outputs of more complex, black box AI. At some point, physicians must be able to trust that the AI they are engaging with works and performs as promised, given that, with complex machine learning algorithms, not even its own developers can always easily ascertain where mistakes are made and how the algorithms arrive at certain decisions.

Another complicated scenario that may arise is the question of what risks physicians face if they do not utilize AI, or if, using their best judgment, they disagree with the AI's assessment, and harm results. This presents another complex legal scenario with no solid precedent. Assigning liability to physicians on the basis that they should not depend on AI outputs and must rely on their independent judgment, while also exposing them to penalties for doing so, effectively places them in an untenable, no-win position.

The AMA believes that AI developers should always be accountable for their performance, as physicians are accountable for theirs. Regulatory structures will play an important role in apportioning liability and serve to help provide assurances of safety and performance, which can play a significant role in mitigating risks to patients and enhancing trust in AI. They also set legal standards that a developer must meet to be deemed appropriate for use in providing care. A lack of regulatory standards for AI impacting clinical care results in physicians potentially shouldering the entirety of the risk of harm, should the AI perform poorly and create a situation where a developer is not accountable for the performance of its product. A lack of regulatory oversight, guardrails, or other development standards can increase safety risks and can create additional liability risks when it is noted that physicians are using unregulated tools. For example, the recent final guidance on Clinical Decision Support (CDS) Software outlines that the FDA plans to exercise "enforcement discretion" for a wide range of CDS software, clearly noting that physicians are responsible for independently reviewing and validating the results. It also states that physicians should not rely on the algorithmic outputs as the basis of their decision. While the AMA does not believe that all CDS tools need stringent oversight and appreciates the exercise of enforcement discretion only if the tool offers some level of transparency, the guidance essentially expects physicians to utilize these tools while also creating a situation in which the tools are not necessarily accountable for their performance.

The AMA expects the liability framework to evolve over time as clinical AI-enabled technologies improve and are incorporated into the standard of care. As the standard of care increasingly contemplates the use of AI, liability may become more clearly defined, provided there is broad agreement that AI should be trustworthy and accountable for its performance and potential impact on clinical care decisions. Even with greater clarity regarding the standard of care with respect to AI, we expect legal questions involving the use of AI in clinical care to continue to be complex. Determining causations in medical malpractice cases is already difficult. Including the input of AI into clinical care adds an additional layer of complexity to this already challenging task.

Interoperability to Accelerate AI in Clinical Care

Enhanced interoperability can meaningfully widen market opportunities, fuel research, and accelerate safe AI deployment when it enables *reliable, standards-based access to high quality clinical data across settings* without shifting new workflow, liability, or privacy risk onto physicians and patients. The biggest upside comes from making it easier for physicians to use AI as a native part of clinical care, embedded within electronic health record (EHR) workflows and grounded in trusted, interoperable data, rather than forcing physicians to step outside the EHR into separate tools.

Interoperability is most catalytic where AI depends on longitudinal, computable, and timely data, especially at points of care transition and high administrative friction. Priority data types include problem lists/diagnoses, medications, allergies, labs and vitals, imaging reports and key findings, procedures and care plans, referrals and consult notes, discharge summaries, and relevant outcomes data. These data are available from both inside the four walls of the clinic and from wearables, remote patient monitoring, and hospital-at-home environments.

Equally important are administrative and operational data that can drive burden and delays: prior authorization status and payer coverage rules. Application programing interface (API) enabled access can reduce redundant documentation and enable AI to generate more context aware recommendations that fit clinical workflows.

Fast Healthcare Interoperability Resources (FHIR) based APIs can be the practical bridge between EHRs and trustworthy AI *if* they are implemented consistently, support writeback where appropriate, and are paired with clear governance and accountability. HHS should prioritize an ecosystem in which:

- **Patients** can access and share their data through standardized APIs with meaningful controls and transparency.
- **Physicians and care teams** can access AI relevant data without manual chart abstraction and can integrate AI tools into clinical workflow with minimal clicks and redocumentation.
- **AI developers** can compete on performance and safety (not bespoke EHR integrations) by building against consistent FHIR profiles and implementation guidance, reducing integration costs and vendor lock in.

Interoperability must move beyond "data movement" to "data meaning." **HHS should emphasize consistent use of standardized, clinically validated code sets and terminologies so AI models train and operate on comparable inputs across settings.** As examples, core clinical concepts should be expressed in widely adopted standards (e.g., diagnoses/problems, labs, medications, procedures) and mapped in ways that are auditable and maintain clinical fidelity. Without strong semantic standardization, AI performance will vary unpredictably across sites, undermining physician trust and increasing patient risk.

Expanded API access increases the attack surface and magnifies the risk of inappropriate secondary use of sensitive health data, especially when third parties may not be regulated under The Health Insurance Portability and Accountability Act in the same way as covered entities. **HHS should caution against interoperability policies that are too loose or overly deferential to developers, and instead pair data access with enforceable protections.** Needed are strong security by default expectations for API implementations, meaningful patient controls, limits on data use beyond what patients authorize, and clear accountability for downstream entities that receive data. FTC and OCR enforcement should be leveraged to deter unfair/deceptive data practices and to ensure privacy and security protections remain credible as data flows increase. **HHS should consider adaptations to the Health Information Technology for Economic and Clinical Health Act's Business Associate Agreement policies to include baseline transparency, safety, and security requirements for all third-party AI tools with access to protected health information.**

Clinical care infrastructure

Independent practices operate with lean staffing and limited information technology capacity, yet they are expected to navigate increasingly complex health care administration rules, quality reporting programs, documentation requirements, and multi-payer administrative workflows. When administrative overhead expands, the immediate consequences are predictable: less time with patients, delayed services, and reduced capacity to take new patients, particularly in rural communities.

Properly deployed, AI can help practices spend less time on administrative tasks that detract from delivering care. **The Administration can accelerate this transformation by aligning federal policy levers around three goals:**

1. Make AI integrate into routine workflows (not separate portals or "shadow processes" that add clicks).
2. Enable competition on performance and burden reduction by reducing integration barriers and developer lock-in.
3. Protect trust with baseline privacy, security, and transparency expectations proportionate to risk.

AI has the potential to reduce burden in several high friction areas that consume disproportionate time in independent practices today. For example, AI enabled documentation support can help reduce duplicative data entry and improve accuracy and compliance without encouraging "note bloat" that obscures clinical meaning. Similarly, AI can streamline eligibility and benefits verification by reducing the back and forth required to confirm coverage details, improving point of care clarity for both patients and staff. AI can lessen manual chart abstraction and rework by converting routine clinical documentation into structured, reusable elements that support quality reporting requirements with fewer additional steps.

AI driven burden reduction is only possible with consistent, computable access to clinical and administrative data elements. Enhanced interoperability, especially through reliable, standards-based API driven exchange can lower integration costs, reduce reliance on bespoke interfaces, and enable more innovators to compete on performance and measurable workflow value. To realize these benefits for independent practices, **HHS should encourage EHR developers and other ecosystem participants to provide dependable, consistent API access that supports practice facing workflows (not only patient facing use cases), and to prioritize implementation consistency so practices are not forced into costly custom builds.**

AI cannot be safe, effective, or trusted if it is built on inconsistent or low integrity data. For physicians, the reliability of an AI recommendation is only as strong as the fidelity of the clinical information feeding it—making trusted, standardized medical coding and terminology foundational infrastructure, not a technical afterthought. **HHS should therefore elevate semantic standardization as a core requirement for responsible AI.** AI training, use, and outputs must be grounded in standardized clinical concepts and code sets and be auditable for clinical fidelity, while driving alignment across EHRs, payers, and clinical decision-support systems so that "meaning" remains consistent as data moves through the health care ecosystem.

Independent physician practices often lack the bargaining leverage to negotiate robust contractual protections with developers and other third parties, even though those entities frequently control the system design, data handling, and security practices that determine real world risk. In that context, a purely market-based approach can inadvertently shift accountability onto physicians by default—exposing practices to privacy, security, and compliance fallout tied to technology they do not fully control. Further, when trust in data security and privacy is degraded, the flow of information halts. **HHS can help correct this imbalance by using clear guidance and an appropriate enforcement posture. Entities building and operating AI systems must carry privacy and security obligations and accountability.**

Prior Authorization

Prior authorization is one of the most significant drivers of physician burden and of delays in care. AI can be used constructively to streamline documentation, improve submission quality, and accelerate approvals. However, the AMA remains concerned about payer use of automated systems, including AI enabled tools, that increase inappropriate denials, obscure the rationale for decisions, or create barriers to timely, medically necessary care.

When automated or AI enabled tools influence prior authorization approvals or denials, physicians and patients need clear, actionable explanations—not opaque outputs that make decisions effectively unreviewable. Prior authorization should not devolve into "black box" process where physicians cannot understand what drove an adverse determination or what information is needed to resolve it. Accordingly, physicians should expect meaningful transparency about the basis for adverse determinations, including the key clinical or administrative factors that contributed to the outcome, and ensure that decision rationales are delivered in a form that works in real clinical workflows. Any AI enabled recommendation that indicates limitations or denials of care, at both the initial review and appeal levels, must be automatically referred to a physician for review.

The critical policy question is not whether AI should be used in prior authorization, but under what conditions it can be deployed without undermining patient protections, due process, or physician trust. **The Administration should advance a set of common-sense expectations that distinguish AI that speeds appropriate approvals from AI that scales inappropriate denials.** To effectively mitigate harms associated with prior authorization, HHS' policies should address several objectives:

- AI can support prior authorization workflow, but adverse coverage decisions must not be fully automated. Denials should require timely review by a qualified, licensed physician, with escalation pathways to protect patient safety when care may be delayed.
- If AI influences coverage decisions, payers must be able to demonstrate real world accuracy and clinical appropriateness through ongoing monitoring for errors, inappropriate denial patterns, and bias. Independent auditing and corrective action must be deployed when AI inappropriately limits patients' access to care.
- Standardized, API driven interoperability should streamline prior authorization end to end within the EHR. APIs should reduce fragmented documentation, eliminate payer specific variation, and rework.
- As API enabled data access expands in prior authorization, privacy, security, and limits on downstream use must scale with it. FTC and OCR enforcement and clear chain of custody accountability must be established so expanded access does not increase breach risk or undermine patient and physician trust.

Payment for AI-Enabled Technologies

The AMA commends the Administration on its increasing focus on developing pathways to payment for emerging digital health technologies. Digital health technologies, ranging from digital therapeutics to AI-enabled diagnostics and from augmentative to fully autonomous systems, are evolving at a rapid pace, with increasing interest from physicians in integrating these technologies into their practice. However, for these tools to ultimately become available for patient care, they need a clear, consistent pathway to payment.

The AMA has long held that any digital health technology proposed for payment should meet the goals of the quadruple aim: improving the patient experience, lowering health care costs, improving physician wellbeing, and improving health outcomes. We have appreciated the HHS' focus on ensuring that the safety and efficacy of AI is a priority when considering payment for AI-enabled technologies. Given that regulatory updates are lagging behind the rapid pace of innovation, ensuring that payment systems prioritize safe and effective AI-enabled systems can play a key role in helping enhance patient and provider trust while encouraging broader adoption of emerging and innovative technologies.

While the AMA strongly supports efforts to develop pathways to payment for high-quality, high-value emerging technologies, the costs of these technologies raise concern over the potential negative impact on the Medicare physician fee schedule (MPFS). AI-enabled technologies can be expensive and as an increasing number of new systems come to market, the costs to the Medicare systems could prove to be very significant. Due to budget neutrality rules, paying for these technologies on the MPFS would likely be unsustainable over the long term. Because of this, the AMA strongly recommends that payment for digital health and AI-enabled technologies be paid outside of the MPFS. A new benefit category or other alternative approach whereby digital health tools and other AI-enabled technologies are paid separately from the MPFS would preserve payment for physician services while seeking to provide a clear, consistent pathway to payment. The unique nature of these technologies would appear to warrant a new payment system tailored specifically to the needs of developers, physicians, and patients when considering integration of these tools into clinical practice. This would be analogous to CMS' approaches to physician-administered drugs paid under Part B and to payment of durable medical equipment. While the AMA recognizes the limitations on CMS' authority to create new benefit categories, we urge the Agency to carefully consider how broad payment of increasing numbers of AI technologies may impact payment for physician services on the MPFS.

Unfortunately, physicians have seen the unintended consequences of paying for high-cost disposable supplies in the practice expense methodology as these services have grown in price and in number. Currently, there are 94 medical supply items embedded in codes with a purchase price of more than $500. These high-cost medical supplies represent $1.53 billion in direct costs and 21 percent of all direct practice expense medical supply costs in the non-facility setting. These supplies are paid for at the expense of other specialties, and we appreciate that CMS is seeking input on how to price and pay for these services. The AMA is reiterating that they should be priced annually and paid outside of the MPFS. We have concerns that the same redistributive trend could emerge if digital health technologies are paid within the MPFS.

Coding for AI-Enabled Technologies

The AMA supports HHS' objective to accelerate the responsible adoption of AI and other digital technologies in clinical care, while fostering public trust, reducing uncertainty that impedes innovation, and aligning incentives to improve outcomes, reduce burden, and lower costs. The AMA believes these goals are best achieved when digital innovation is grounded in clinical reality, governed through transparent, clinical expert-led processes, and integrated into sustainable coding and payment pathways.

A central mechanism for advancing these objectives is the ongoing evolution of the Current Procedural Terminology® (CPT®) code set. Through the work of the CPT Editorial Panel, CPT is being modernized to support digitally enabled value-based care models that are increasingly outcomes-focused, technology-enabled, and delivered by multidisciplinary teams. CPT already underpins many value-based care arrangements by supporting patient attribution, risk stratification, quality measurement, and longitudinal analysis. Building on this foundation, the Panel has established a dedicated Value-Based Care Workgroup to evaluate how CPT can better reflect episode-based and digitally enabled care, operating through an open and transparent process that actively seeks input from physicians, payers, innovators, federal agencies, and other stakeholders.

In parallel, the CPT Editorial Panel is addressing the rapid emergence of clinical AI by developing a distinct coding framework that accurately describes algorithmic services as part of the patient care pathway. Recognizing that existing payment systems often rely on ad hoc crosswalks that are not scalable or sustainable, the Panel's Digital Medicine Coding Committee is advancing a new category of CPT codes, Clinically Meaningful Algorithmic Analyses. These codes are intended to describe complete, codifiable clinical services in which algorithmic output meaningfully informs diagnosis or treatment, while clearly distinguishing machine-performed work from the interpretive work of physicians and other qualified health care professionals.

Beyond defining algorithmic services themselves, the Panel recognizes that the increasing use of AI will have broader implications for physician work and for how clinical services are described across the CPT code set. Accordingly, the Panel has prioritized evaluating potential impacts on physician work, identifying coding gaps, and assessing how existing codes may need to evolve as clinical practice changes. These considerations will inform future deliberations and guide the next phase of CPT modernization to ensure continued clinical accuracy and relevance.

Because CPT is embedded across electronic health records, payer systems, quality programs, and federal health care infrastructure, updates to the code set provide a scalable mechanism for integrating digitally enabled value-based care and clinical AI across the health system. While the CPT Editorial Panel has taken a leadership role in developing a clinically grounded and transparent reporting framework for algorithmic services, the AMA recognizes the essential role of HHS in establishing sustainable pathways for coverage and payment. The AMA looks forward to continued collaboration with HHS and other stakeholders to ensure that advances in digital health and AI translate into patient benefit, physician adoption, and a more efficient, trusted, and sustainable U.S. health care system.

Thank you for considering our recommendations. The AMA remains committed to working with the Administration to advance our shared goals. Please reach out to me directly at 312-464-5288 or John.Whyte@ama-assn.org if you have questions or need further information.

Sincerely,

John Whyte, MD, MPH